

Recent trends in the adoption of complementary and alternative medicine: Usage Patterns across age groups and disease categories (2025-26)

Statistics Department

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Group 2, Primary data analysis

by

Pratanu Chowdhury 401

Asmit Kumar Mukherjee 412

Souparno Sikdar 413

Tuhin Debnath 429

Aarushi Datta 449

Soumili Banerjee 458

Kabeer Hussain 465

1. INTRODUCTION

1.1 Background

In recent years, healthcare practices have evolved as individuals increasingly seek diverse approaches to maintain and improve health. Complementary and Alternative Medicine (CAM) — including herbal remedies, yoga, meditation, acupuncture, and naturopathy — has gained growing attention worldwide, used either alongside conventional treatments or as an alternative depending on individual needs.

The rise in CAM usage is driven by greater awareness of holistic health, the preference for natural and preventive care, and wider access to information on alternative therapies. Cultural traditions and concerns about lifestyle-related diseases, mental health, and long-term medication use also contribute to its adoption.

Use of CAM varies by demographic factors such as age, gender, health status, and personal attitudes, with some individuals using it for wellness and prevention, and others for managing specific conditions. This study analyzes primary data from 2025–2026 to examine trends in CAM adoption, focusing on usage patterns across age groups and attitudes toward alternative therapies, to better understand its integration into modern healthcare.

1.2 Research Questions

This study aims to answer the following questions:

1. How do demographic factors such as age, education, and income influence the use and expenditure on CAM?
2. Are individuals with chronic conditions more likely to use CAM?
3. Does avoidance of prescription medicines influence CAM usage?
4. Which health conditions have the highest CAM usage, and how does usage frequency vary across these conditions?
5. How do perceptions of safety, effectiveness, and trust influence CAM usage?
6. What prevents non-users from adopting CAM?

2. Data Description

2.1 Source and Sample

The dataset consists of survey responses collected from students and their family members, who reported whether they use alternative medicine, the frequency of use, disease categories, and perceived effectiveness.

2.2 Variable and Types

<u>Variable</u>	<u>Type</u>	<u>Description</u>
Age	Numeric	Age in years
Gender	Categorical	Gender of respondent
Highest Education Level	Ordinal	Highest education received by respondent
Monthly Household Income	Ordinal	Income group the respondent belongs to
Diagnosis of Chronic conditions in the past 12 months	Binary	Does the respondent has any chronic condition?
Use of Prescription medication	Binary	Is the respondent taking any current prescribed medicine?
Use of Alternative medicine	Binary	Does the respondent use alternative medicine?
Reasons for not using Alternative medicine	Descriptive	If not, why not?
Type of Alternative medicine	Categorical	If yes, what kind?
Primary reason for using Alternative medicine	Descriptive	If yes, why?
Monthly Spending on Alternative medicine	Ordinal (Classes of Numeric)	How much does the respondent spend on alternative medicine?
Informing healthcare provider	Binary	Does primary healthcare provider know about using alternative medicine?
Effectiveness rating	Ordinal	How effective does the respondent think the medicine is?
Disease for using alternative medicine	Categorical	Using for which kind of disease?
Frequency of Use	Ordinal	How often is the medicine taken?
Safety perception	Ordinal	How safe does the respondent think alternative medicine is?
Trust in practitioner	Ordinal	Does the respondent trust in alternative medicine?
Can alternative medicine replace conventional medicine	Ordinal	Does the respondent think alternative medicine can replace conventional ones?

Source of information about alternative medicine	Nominal	How did the respondent get to know about alternative medicine?
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3. Methodology

3.1 Data Collection

- Data was collected via surveys administered to students and their family members.
- Respondents provided information on:
 - Whether they use alternative medicine
 - Frequency of use (Daily, Weekly, Monthly, Rarely)
 - Disease categories treated with alternative medicine
 - Perceived effectiveness (1–5 scale)
 - Demographics: Age, Gender, Participant type (Student / Family Member)

3.2 Data Cleaning and Preprocessing

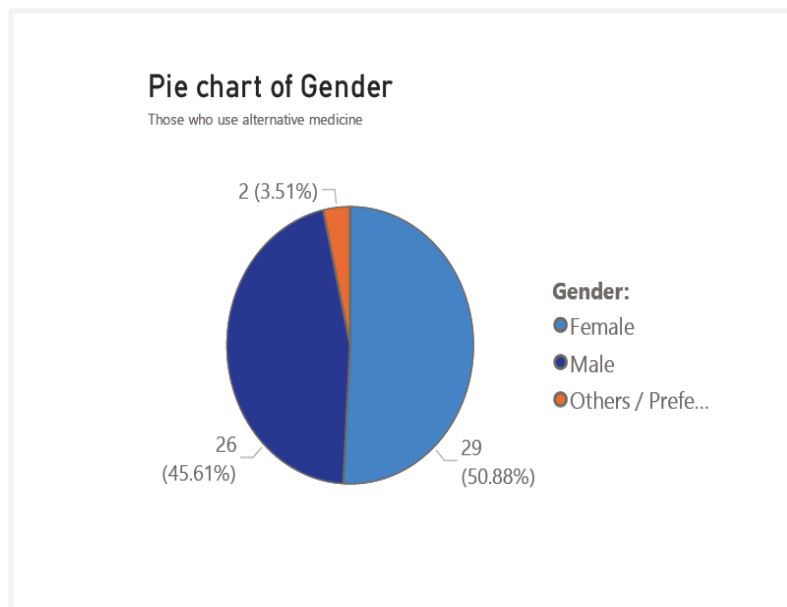
- Multiple disease entries in a single response were split into separate rows.
- Missing or empty entries were removed.
- Ambiguous entries (“something”) were grouped under “Others”.
- Frequency categories were standardized to Daily, Weekly, Monthly, Rarely.
- SMOTE (Synthetic Minority Over-sampling Technique) was applied to balance the dataset for binary outcomes alternative medicine use, to ensure that models like logistic regression were not biased toward the majority class.

3.3 Analytical Methods

1. Exploratory Data Analysis (EDA)
 - To summarize patterns in the data and identify trends.
 - Included pie charts (e.g., gender distribution), faceted bar plots (frequency of use by disease), and boxplots (perceived effectiveness by disease).
2. Logistic Regression
 - Used to model the relationship between binary outcomes (e.g., whether a person uses alternative medicine) and predictors like age, gender, or disease category.
 - Helps identify significant factors influencing the likelihood of alternative medicine use.
3. Contingency Tables and Measures of Association
 - Used to examine relationships between categorical variables, such as Chronic Disease vs. alternative medicine use, avoidance of prescription medicines vs alternative medicine etc.
 - Measures such as Chi-square tests and odds ratios were used to quantify the strength and significance of associations.

4. Explanatory Data Analysis

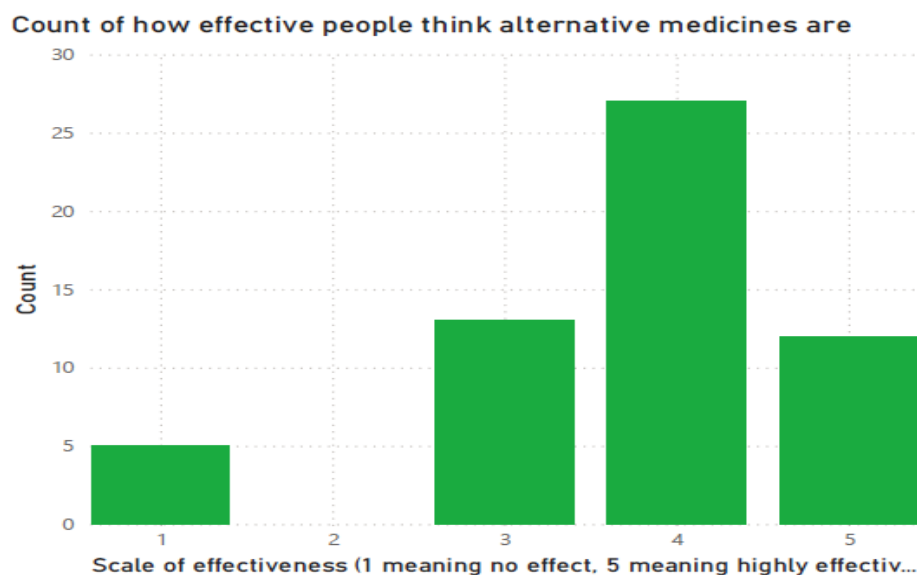
5. Figure 4.1 Gender Distribution



Interpretation:

The bar chart shows that 45.61% of respondents are male and approximately 51% are female. This reflects a marginally higher representation of females in the sample, although the gender distribution is fairly balanced and unlikely to significantly bias the analysis.

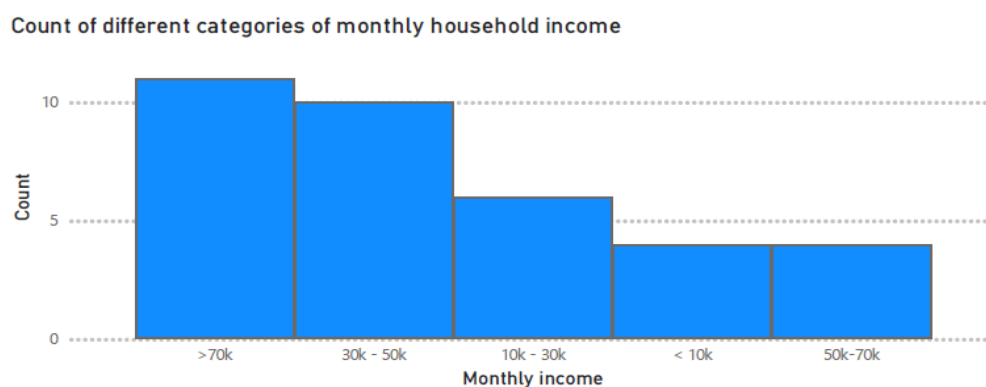
Figure 4.2 Respondents' Ratings of Alternative Medicine Effectiveness (Scale 1–5)



Interpretation:

The effectiveness ratings are concentrated around 4, which has the highest frequency, whereas 1 has the lowest. This suggests a generally positive perception of alternative medicine among respondents. However, the lower frequency of rating 5 indicates that while respondents tend to view it as effective, full confidence in its effectiveness is not widespread.

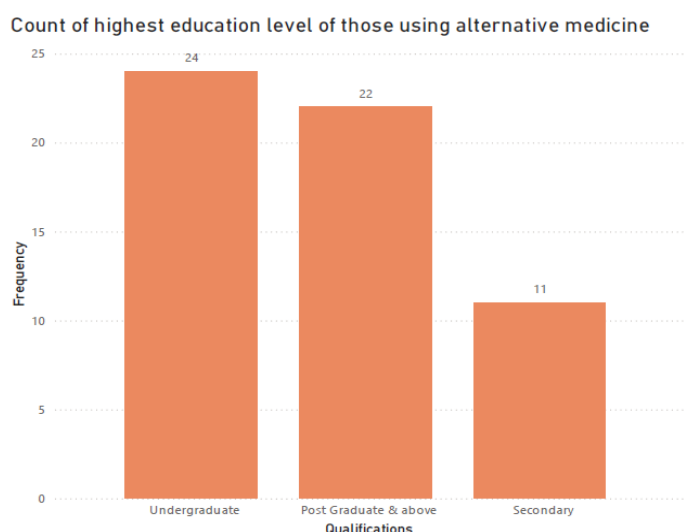
Figure 4.3 Distribution of Monthly Household Income of Respondents



Interpretation:

The distribution shows that a larger proportion of respondents belong to higher income groups, with the above ₹70,000 category having the highest frequency, followed by the ₹30,000–₹50,000 group. In contrast, lower income categories, particularly below ₹10,000, are underrepresented. This indicates that the sample is skewed towards relatively higher-income individuals, which may influence patterns of healthcare choices and expenditure.

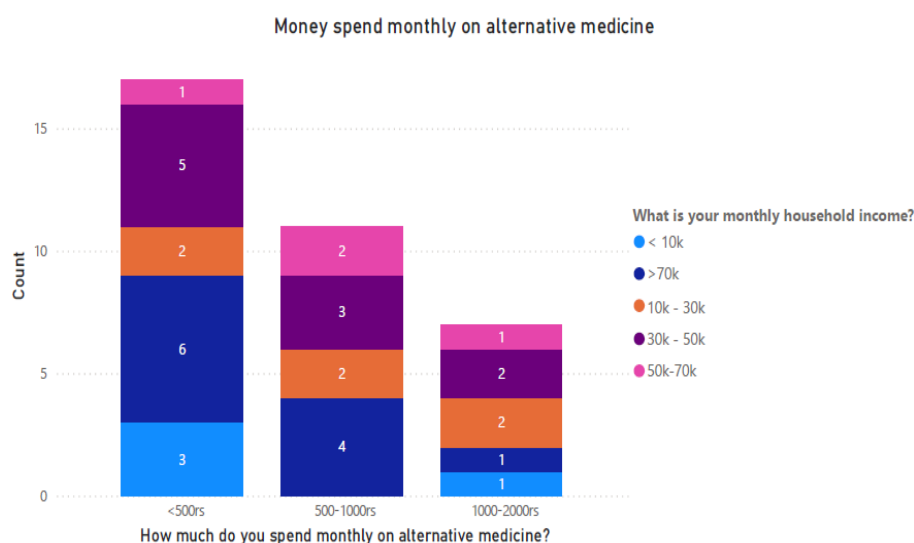
Figure 4.4 Educational Qualification of Respondents



Interpretation:

The chart shows that the majority of respondents are undergraduates (24), followed by those with postgraduate education and above (22), while respondents with secondary education (11) form the smallest group. This indicates that the sample is skewed towards individuals with higher educational qualifications. Such a distribution may influence the findings, as more educated respondents may have greater awareness and exposure to different healthcare practices, including alternative medicine.

Figure 4.5 Monthly Expenditure on Alternative Medicine Across Income Groups



Interpretation:

The chart illustrates the distribution of monthly expenditure on alternative medicine across different income groups. It can be observed that the majority of respondents, particularly from higher income groups (above ₹70,000 and ₹30,000–₹50,000), tend to spend less than ₹500 per month. As expenditure increases to the ₹500–₹1000 and ₹1000–₹2000 ranges, the number of respondents decreases across all income categories.

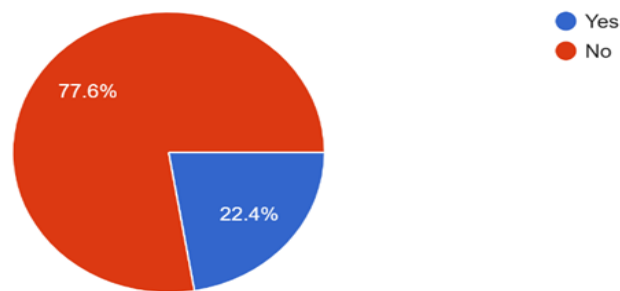
This suggests that, irrespective of income level, most individuals prefer to keep their spending on alternative medicine relatively low. However, higher income groups are still more represented across all expenditure categories, which may indicate greater affordability and willingness to spend. At the same time, lower income groups show comparatively lower participation in higher expenditure ranges, possibly due to budget constraints or prioritization of essential expenses.

6. Handling Imbalanced Data

5.1 Imbalanced Data

An imbalanced dataset refers to a situation where the number of observations in one category of the dependent variable is significantly higher than in the other. In this study, the variable representing the *use of alternative medicine* exhibited class imbalance

Do you use alternative medicine ?
255 responses



The above pie chart represents the distribution of respondents based on their use of alternative medicine. Out of a total of 255 responses, 77.6% of individuals reported not using alternative medicine, while only 22.4% reported using it.

This indicates a clear imbalance in the dependent variable, with the majority of observations belonging to the “No” category. Such a skewed distribution suggests that the dataset is heavily dominated by non-users of alternative medicine, while users form a relatively smaller proportion.

5.2 Need for Addressing Imbalance

The presence of class imbalance can lead to biased results, as statistical models tend to be influenced more by the majority class. This reduces the reliability and accuracy of the analysis, particularly when studying relationships between variables. Therefore, it is essential to correct for imbalance before proceeding with further statistical techniques.

5.3 Synthetic Minority Oversampling Technique (SMOTE)

To address this issue, the Synthetic Minority Oversampling Technique (SMOTE) was applied. SMOTE is a resampling method that generates synthetic observations for the minority class based on existing data points. Instead of duplicating observations, it creates new data points

by interpolating between nearby minority class instances. This approach helps in improving the representation of the minority class while preserving the structure of the dataset.

However, SMOTE relies on distance calculations, which require all variables to be numerical. This creates a problem when the dataset contains categorical variables, such as gender, because assigning simple numeric values would introduce artificial ordering and distort distances. To overcome this, one-hot encoding is applied, where each category is converted into separate binary variables, ensuring that no false relationships are introduced. Thus, the correct workflow involves first converting categorical variables into numerical form using one-hot encoding, then applying SMOTE to balance the dataset, and finally fitting the logistic regression model. This process ensures that the model is trained on balanced and properly structured data, leading to more reliable estimation and inference.

This ensures that:

- The minority class is better represented
- The dataset remains structurally consistent
- And our model can learn patterns more effectively

5.4 Outcome

After applying SMOTE, the dataset was successfully balanced with respect to the dependent variable. This ensured that both categories were adequately represented, thereby improving the robustness and reliability of subsequent analyses such as correlation and regression.

Additionally, after the resampling process, the dataset was reorganized to restore categorical variables to their original format. This step was undertaken to maintain interpretability and consistency with the original structure of the data.

7. Logistic Regression Analysis

6.1 Purpose

To examine how age and gender influence the likelihood of using alternative medicine, we performed a binary logistic regression. This analysis allows us to quantify the effect of each predictor while controlling for the others.

6.2 Model Specification

The logistic regression model is given by:

$$\text{logit}(P(\text{Alt_Use} = 1)) = \beta_0 + \beta_1 \text{Age} + \beta_2 \text{GenderMale}$$

Table 6.2 Estimated Coefficients of Logistic Regression Model

Variable	Estimate	Std. Error	z value	p-value	Significance
Intercept	-0.4389	0.2778	-1.580	0.1141	Not Significant
Age	0.0269	0.0083	3.252	0.0011	** Significant
Gender (Male)	-0.7666	0.2102	-3.647	0.0003	*** Highly Significant

Interpretation

The results of the logistic regression provide important insights into how age and gender influence the likelihood of using alternative medicine.

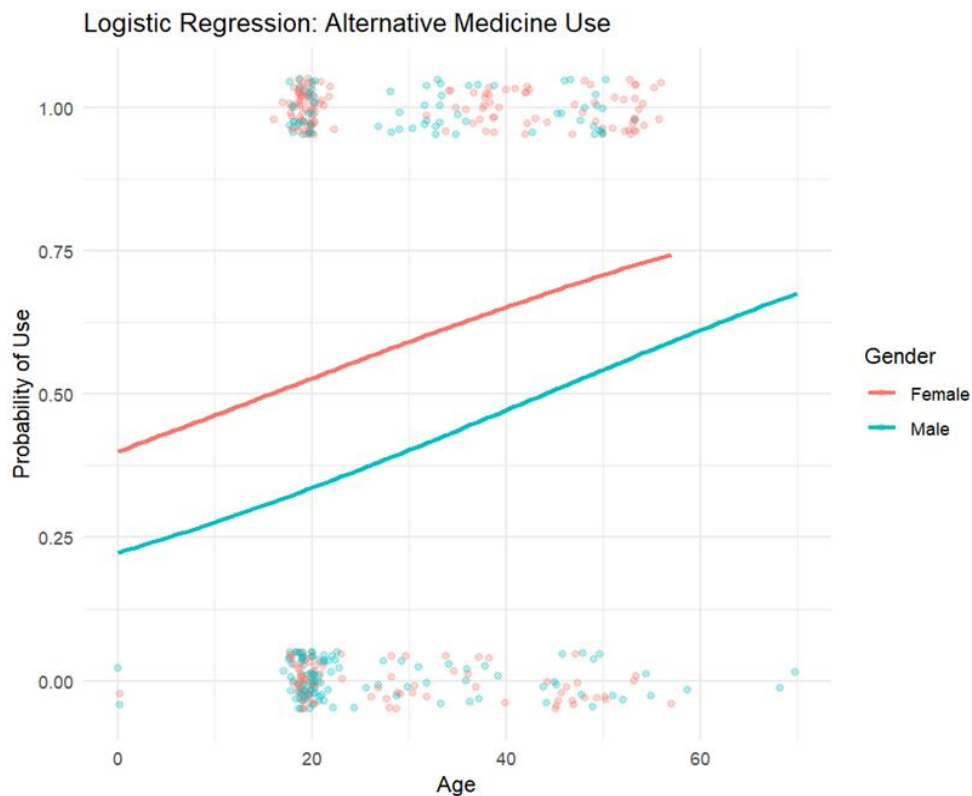
The coefficient for age is positive and statistically significant, indicating that age has a meaningful effect on alternative medicine use. Specifically, for each one-year increase in age, the log-odds of using alternative medicine increase by 0.027. In terms of odds, The odds of using alternative medicine for individuals who are one year older are 1.027 times the odds for individuals who are one year younger, holding other variables constant which corresponds to an approximate 2.7% increase in the odds of using alternative medicine for every additional year of age, holding gender constant. This suggests a gradual but consistent trend where older individuals are more inclined towards alternative medicine, possibly due to greater health concerns, chronic conditions, or increased exposure to traditional remedies over time.

The coefficient for males is negative and highly statistically significant, indicating that gender plays a substantial role in determining alternative medicine use. Compared to females (the reference category), males have significantly lower log-odds of using alternative medicine. The corresponding odds ratio (≈ 0.46) implies that males have 46% of odds of using alternative medicine than females, that is, about 54% lower odds of using alternative medicine than females, controlling for age. This suggests that females are considerably more likely to engage in alternative medicine practices, which may reflect differences in health-seeking behaviour, awareness, or openness to non-conventional treatments.

The intercept represents the log-odds of using alternative medicine by a female when age is zero. However, the practical interpretation is not meaningful in this context because age cannot be zero.

Overall, the model indicates that age and gender are both important determinants of alternative medicine use. The positive effect of age and the strong negative effect for males highlight clear demographic differences in usage patterns.

Figure 6.1 Logistic Regression Plot



Interpretation:

The logistic regression plot shows the predicted probability of alternative medicine use as a function of age for males and females. The probability of use increases steadily with age for both genders, indicating a positive relationship between age and alternative medicine adoption.

At all ages, the predicted probability for males is consistently lower than that for females, reflecting the negative coefficient associated with the male category in the regression model. This suggests that males are less likely to use alternative medicine compared to females, holding age constant.

8. Association Analysis (Effects of Medical History)

7.1 Purpose

To examine relationships between categorical variables, contingency tables and statistical measures of association were employed.

7.2 Association Between Chronic Condition and Alternative Medicine Usage

7.2.1 Contingency Table

Table 7.2.1 Chronic Condition vs. Alternative Medicine Usage

CHRONIC CONDITION	NO (USAGE)	YES (USAGE)	TOTAL
NO	169	167	336
YES	29	31	60
TOTAL	198	198	396

7.2.2 Comparison of Adoption Rates

Table 7.2.2: Adoption Rate of Alternative Medicine by Condition Status

CHRONIC CONDITION	USE ALTERNATIVE MEDICINE (%)
NO	49.7%
YES	51.7%

Interpretation:

Individuals diagnosed with chronic conditions show a slightly higher adoption rate (51.7%) of alternative medicine compared to those without chronic conditions (49.7%). However, the difference is minimal (approximately 2%), indicating only a marginal inclination towards alternative medicine among those with chronic conditions. This suggests that the presence of a chronic condition may not be a strong determinant of alternative medicine usage in this dataset.

7.2.3 Measure of Association

Yule's Coefficient (Q) measures the strength of association between two binary variables on a scale from -1 to +1.

Yule's Coefficient (Q): 0.04

Interpretation:

A Yule's Coefficient value of 0.04 indicates a very weak positive association between chronic condition status and the use of alternative medicine. This implies that the two variables are nearly independent, and the presence of a chronic condition does not significantly influence the likelihood of using alternative medicine in this dataset.

7.3 Association Between Prescription Medicine Usage and Alternative Medicine

7.3.1 Contingency Table

Table 7.3.1: Prescription Medicine Usage vs. Alternative Medicine Usage

PRESCRIPTION USAGE	ALT MEDICINE (NO)	ALT MEDICINE (YES)	TOTAL
NO	146	113	259
YES	52	85	137
TOTAL	198	198	396

7.3.2 Hypotheses

- Null Hypothesis (H_0): Prescription medicine usage and alternative medicine usage are independent.
- Alternative Hypothesis (H_1): There is a significant association between the two variables.

7.3.3 Expected Frequencies

Expected frequencies are calculated under the assumption of independence:

$$\text{Expected Frequency} = \frac{\text{Row Total} \times \text{Column Total}}{\text{Total Frequency}}$$

- No Prescription & Yes Alt: 129.5
- Yes Prescription & Yes Alt: 68.5
- Yes Prescription & No Alt: 68.5
- No Prescription & No Alt: 129.5

7.3.4 Chi-Square Test Statistic

Since the attributes have binary classification and the sample size $n \sim 400$ which is quite large, we approximate the distribution of the observed frequencies to be poisson with mean the same as the expected frequency

Observed Frequency = O_i (for all i from 1 to 4)

Expected frequency = E_i (for all i from 1 to 4)

So $O_i \sim \text{Poi}(E_i)$ where O_i and E_i are the observed and expected frequencies for the i th cell.

Now assuming the survey was conducted a large number of times, and we get new values of O_i for each case, we can make the following assumption by Central Limit Theorem,

By CLT,

$$Z_i = (O_i - E_i) / \sqrt{E_i} \sim N(0, 1) \quad \forall \quad i \in [1, 4], i \in N$$

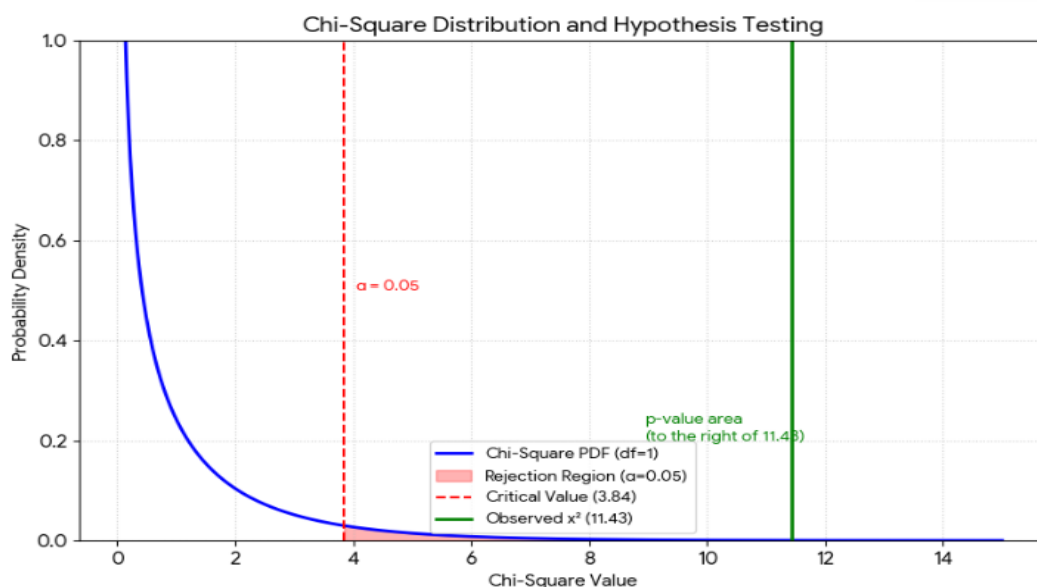
$$\chi^2 = \sum \frac{(O_i - E_i)^2}{E_i}$$

Therefore, $\chi^2 = Z_1^2 + Z_2^2 + Z_3^2 + Z_4^2$, where $\chi^2 \sim \chi^2_{(1)}$ i.e. with one dof

In this case,

Chi-Square Statistic (χ^2): 11.43, p-value: 0.00072

Figure 7.3.4 Chi-Square Distribution with Rejection Region and Test Statistic



Interpretation:

The results suggest that individuals taking prescription medicines are more likely to use alternative medicine (approximately 62%) compared to those who are not taking prescription medicines (43.6%). This indicates that alternative medicine is more commonly used as a complementary treatment alongside conventional medicine rather than as a substitute.

The relatively high chi-square value further supports the presence of a meaningful association between the two variables.

In the graph -

- The Blue Curve (Chi-Square Distribution): This represents the theoretical distribution of the Chi-Square statistic if the null hypothesis (H_0) were true. For 1 degree of freedom ($df=1$), the curve starts high and drops quickly.
- The Red Shaded Area ($\alpha = 0.05$): This is the Rejection Region. It begins at the Critical Value (3.84). If our calculated statistic falls in this area, we reject the null hypothesis.
- The Red Dashed Line (Critical Value): At 3.84, this is the "Upper Alpha Point." Any result to the right of this line is considered statistically significant at the 5% level.
- The Green Vertical Line (Observed $\chi^2 = 11.43$): This is our actual result.

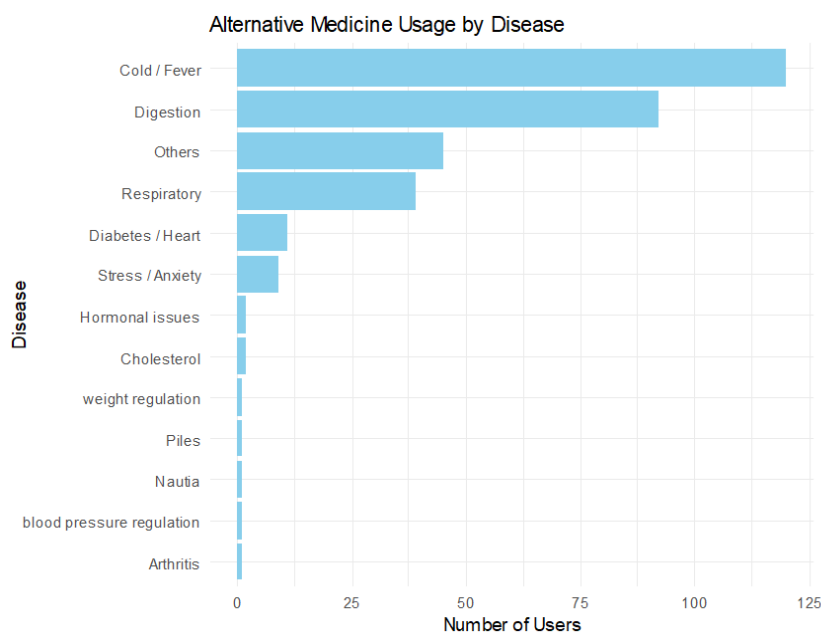
7.3.5 Conclusion of Hypothesis Test

The decision rule is: At a 5% level of significance ($\alpha = 0.05$), we reject the null hypothesis if the p-value is less than 0.05. Since the p-value (0.00072) is significantly less than 0.05, the null hypothesis is rejected. This indicates a statistically significant association between prescription medicine usage and alternative medicine usage.

9. Reliance on Alternative Medicine Across Disease Categories

We examined how reliance on alternative medicine varies across different disease categories to identify which health conditions show the greatest dependence on these medicines.

Figure 8.1 Alternative Medicine Usage By Disease



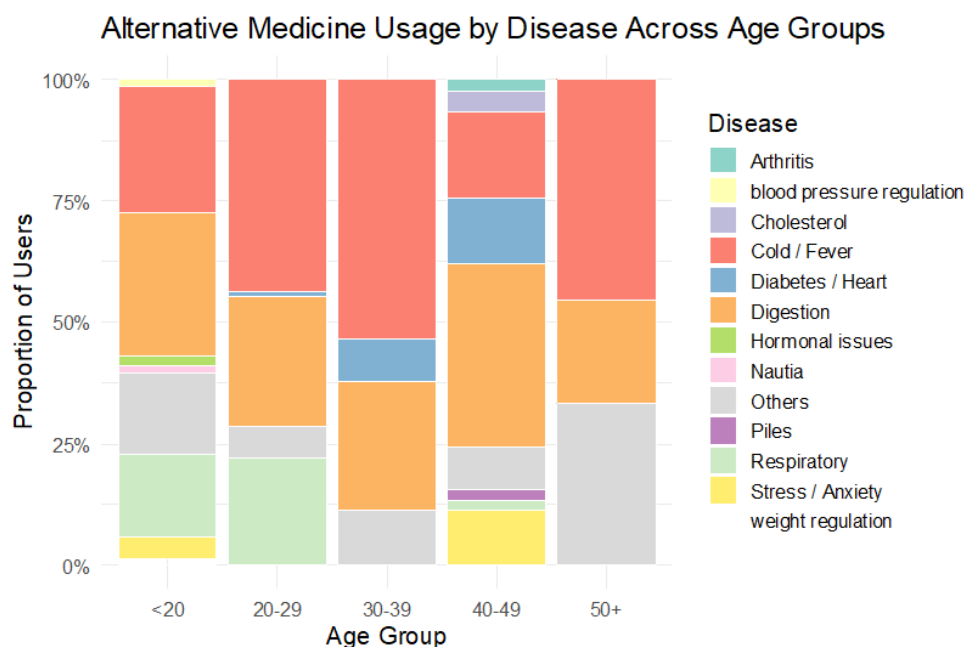
Observations

Among users of alternative medicine, Cold / Fever and Digestion problems are the most commonly treated conditions. Respiratory issues also see moderate use. Specialized or chronic conditions like Diabetes / Heart, Cholesterol, or Arthritis are less commonly treated with alternative medicine. This indicates that alternative medicine is most relied upon for common, recurrent health issues.

10. Age-wise preference of alternative medicine

This section explores whether different age groups prefer alternative medicine for different diseases. We analyse the distribution of diseases for which alternative medicine is used across age groups, using both counts and proportions.

Figure 9.1 *Proportion of users of alternative medicine by disease across age groups.*



Each bar represents an age group, and each colour segment shows the proportion of users using alternative medicine for that disease. This visually demonstrates how usage patterns vary with age.

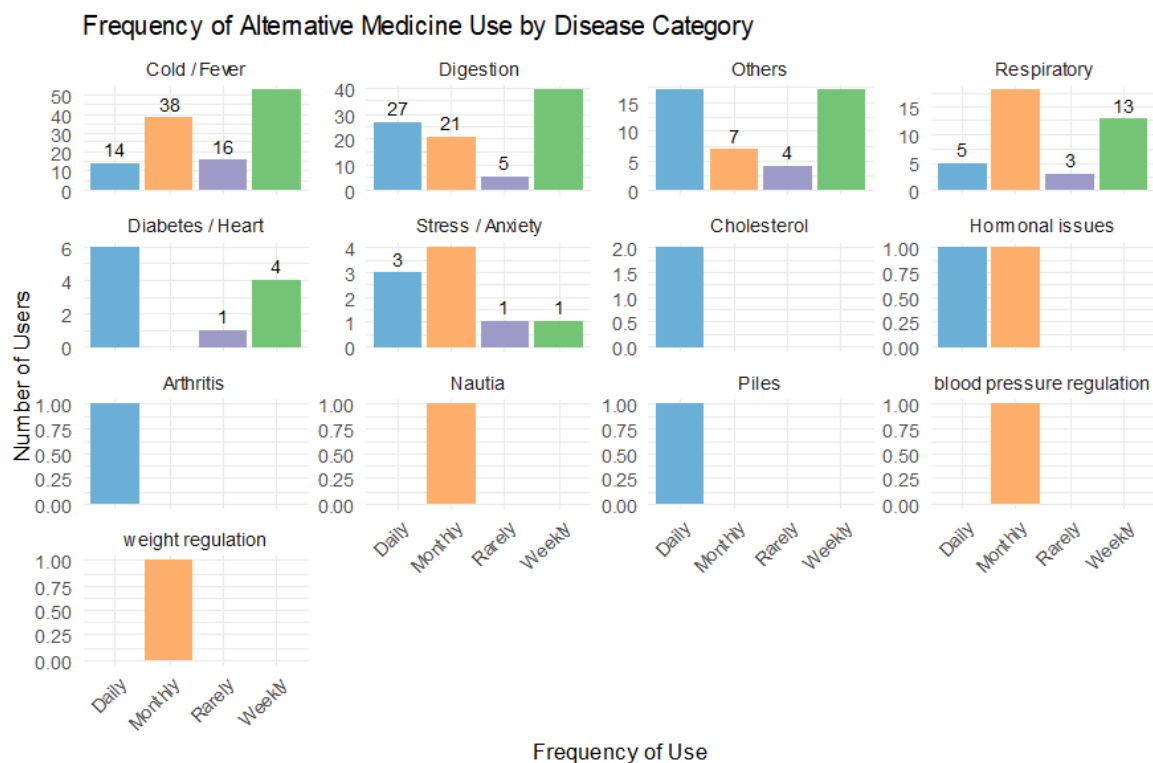
Observations

The chart illustrates the proportion of users relying on alternative medicine for different health conditions across age groups:

- Cold / Fever and Digestion are the most consistently treated conditions across all age groups, indicating that these are the primary reasons people use alternative medicine.
- Older age groups (40+) tend to use alternative medicine more for chronic or long-term conditions, such as Diabetes / Heart, Cholesterol, and Arthritis.
- Younger age groups (<30) show greater usage for Respiratory issues, Blood Pressure regulation, and Hormonal problems, reflecting health concerns more common in early adulthood.
- Stress / Anxiety appears to be an important reason for using alternative medicine among the youngest users (<20) and remains relevant in the 40-49 age group.

11. Variation in the frequency of use across different disease categories

To examine how the frequency of alternative medicine use varies across different disease categories, respondents were classified based on how often they use such treatments (daily, weekly, occasionally, or rarely). This analysis helps identify whether certain health conditions are associated with more regular or infrequent use of alternative medicine.

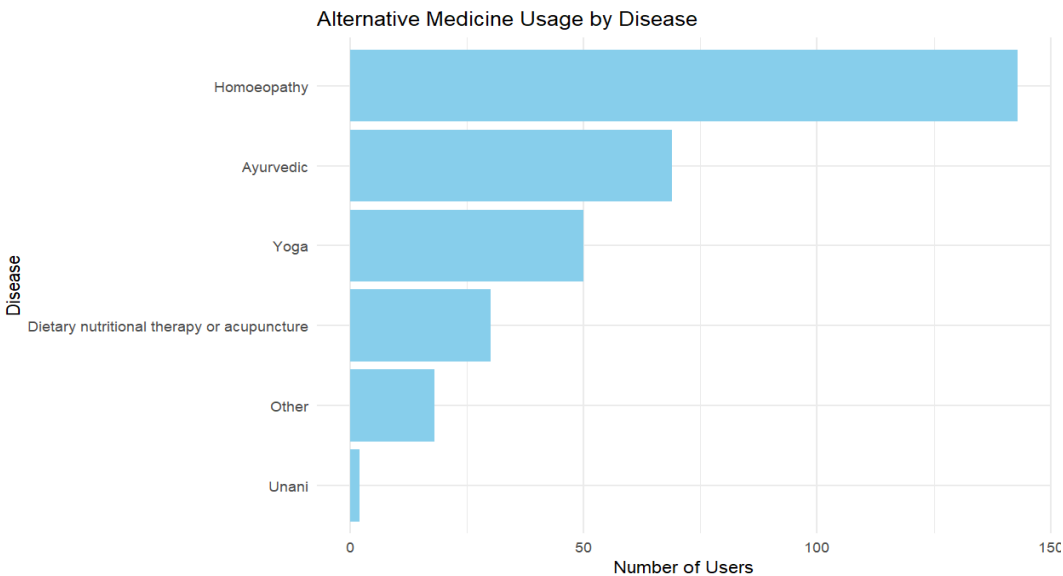


Observations

Patients with long-term conditions like Diabetes/Heart, Cholesterol, Arthritis, and Piles tend to use alternative medicine daily, likely because these conditions require ongoing management. On the other hand, those with short-term conditions like Cold/Fever, Digestion, and Respiratory issues mostly use it weekly or monthly, since these illnesses are temporary.

12. Comparative Usage of Different Alternative Medicine Practices

This section analyzes the comparative usage of different alternative medicine practices to understand the relative preference for various treatment approaches. By evaluating the frequency of use across practices such as Ayurveda, Homeopathy, Yoga, and other therapies, the study seeks to identify dominant trends and variations in healthcare choices among respondents.



Interpretation

The chart shows that homeopathy is the most widely used form of alternative medicine among respondents, with a significantly higher number of users compared to other therapies. Ayurvedic treatments are the second most preferred, followed by yoga, indicating a moderate level of adoption.

In contrast, dietary/nutritional therapy and acupuncture have relatively lower usage, while Unani medicine is the least utilized among all categories. The “other” category also shows limited adoption.

Overall, the results suggest a strong preference for traditional and well-established alternative medicine systems, while less familiar or less accessible therapies tend to have lower usage rates.

13. Conclusion

This study presents a comprehensive analysis of the adoption of Complementary and Alternative Medicine (CAM) using primary data collected from 255 respondents. By combining descriptive analysis with inferential techniques such as logistic regression and chi-square testing, the study provides a nuanced understanding of the factors influencing CAM usage.

The findings indicate that CAM adoption, while limited (22.4%), is not random but systematically influenced by demographic and medical factors. In particular, age and gender emerge as significant predictors. Logistic regression results show that the likelihood of CAM usage increases with age, suggesting that older individuals are more inclined toward CAM, likely due to a higher prevalence of chronic health conditions and greater exposure to traditional remedies. Females are also found to have a higher probability of adoption, reflecting differences in health awareness and health-seeking behaviour.

The study further establishes that CAM is predominantly used as a complementary form of healthcare rather than a substitute. The statistically significant association between prescription medicine usage and CAM adoption confirms that individuals undergoing conventional treatment are more likely to also engage with alternative therapies, indicating an integrative approach to healthcare.

Disease type plays a crucial role in determining usage patterns. Chronic conditions such as diabetes, arthritis, and cholesterol-related issues are associated with regular and sustained CAM usage, often on a daily basis, whereas acute conditions like cold, fever, and digestive issues correspond to occasional or short-term use. This highlights the perceived role of CAM in long-term health management rather than immediate symptom relief.

In terms of modality, Homoeopathy, Yoga, and Ayurveda emerge as the most commonly used forms, frequently in combination, suggesting a flexible and personalized approach among users. Perception analysis reveals that CAM is viewed positively overall, with safety receiving the highest rating, followed by effectiveness, while trust in practitioners remains comparatively lower, indicating a need for improved standardization and professional credibility.

Additionally, awareness and adoption of CAM are largely influenced by informal social networks such as family and friends, rather than formal medical advice. This reliance on non-institutional sources points to a gap in structured healthcare communication and highlights the need for greater involvement of healthcare professionals in guiding CAM usage.

In conclusion, CAM occupies a niche but evolving role in the healthcare landscape, characterized by selective adoption, complementary use alongside conventional medicine, and strong influence from demographic and social factors. The findings emphasize the importance of evidence-based integration, increased awareness, and enhanced regulatory frameworks to support the safe and effective incorporation of CAM into mainstream healthcare systems.

ANNEXURE I

QUESTIONNAIRE

The following questionnaire was used to collect data for the study on Recent Trends in Adoption of Complementary and Alternative medicine.

Section A: Personal Details

1. Age: _____

2. Gender:

☐ Male

☐ Female

☐ Non-binary

3. Highest Education Level:

☐ Primary

☐ Secondary

☐ Bachelor's

☐ Master's

4. Monthly Household Income:

☐ Less than ₹10,000

☐ ₹10,000–₹30,000

☐ ₹30,000–₹50,000

☐ More than ₹50,000

Section B: Medical History

5. Have you been diagnosed with any chronic condition in the past 12 months?

☐ Yes

☐ No

6. If yes, please specify: _____

7. Duration of primary condition:

☐ Less than 1 year

☐ 1–5 years

☐ 5–10 years

☐ More than 10 years

8. Are you currently taking any prescription medications?

☐ Yes

☐ No

9. Do you use alternative medicine?

☐ Yes

☐ No

Section C: Use of Alternative Medicine

10. Types of alternative medicine used:

☐ Ayurvedic

☐ Unani

☐ Homeopathy

☐ Yoga

☐ Dietary/Nutritional therapy or Acupuncture

☐ Others: _____

11. Primary reason for use:

☐ Treat specific condition

☐ General wellness

☐ Side effects of conventional medicine

☐ Others: _____

12. Monthly expenditure:

☐ <₹500

☐ ₹500–₹1,000

☐ ₹1,000–₹2,000

☐ >₹2,000

13. Informed healthcare provider:

☐ Yes

☐ No

14. Effectiveness (1–5):

☐ 1

☐ 2

☐ 3

☐ 4

☐ 5

15. Conditions treated:

☐ Pain/Joints

☐ Stress/Anxiety

☐ Digestion

☐ Diabetes/Heart

- ☐ Cold/Fever
- ☐ Respiratory
- ☐ Others: _____

16. Frequency of use:

- ☐ Daily
- ☐ Weekly
- ☐ Occasionally
- ☐ Rarely

17. If not used, reason:

- ☐ Lack of awareness
- ☐ Cost
- ☐ Safety concerns
- ☐ No belief
- ☐ Doctor advised against

Section D: Beliefs Regarding Alternative Medicine

18. Safety perception (1–5):

- ☐ 1
- ☐ 2
- ☐ 3
- ☐ 4
- ☐ 5

19. Trust in practitioners (1–5):

- ☐ 1
- ☐ 2
- ☐ 3
- ☐ 4
- ☐ 5

20. Can it replace conventional medicine?

- ☐ Yes completely
- ☐ Only as complement
- ☐ For some conditions
- ☐ Not at all

21. Source of information:

- ☐ Doctor
- ☐ Healthcare personnel
- ☐ Family/Friends
- ☐ Advertisements

☐ Others: _____

The form link is as follows:

<https://docs.google.com/forms/d/e/1FAIpQLSc2jvQKlieZAST0XFarggc82qdD-fhoaggfaqpkuhoQPOrlOA/viewform?usp=dialog>

The survey was conducted on various social media platforms like Linked In, Whatsapp and Instagram, with no specific target age group, The majority respondents were in the age group 19-30 with some respondents being around 40 and 50 years. The time-frame of the survey was from February 8, 2026- February 23, 2026 with total respondents amounting to 255.